

UPMC Shadyside

5230 Centre Ave, Pittsburgh, PA 15232

DISCHARGE SUMMARY Document #: DIS-012

Patient & Admission

Name	Elizabeth Harvey
Date of Birth	1975-01-25
Phone	724-555-9763
Address	7738 Leon Underpass Apt. 148 Pittsburgh, PA, 15222
MRN	MRN-433200
Admit Date	2026-05-03
Discharge Date	2026-05-10
Attending	Kelsey Harper, MD
Attending NPI	1939909168

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	JIM514846564

Primary Discharge Diagnosis

HFrEF with acute decompensation

ICD-10: I50.22

Hospital Course

The patient presented to the emergency department with acute dyspnea, orthopnea, and lower extremity edema. Initial vital signs revealed hypertension and tachycardia, with oxygen saturation of 88% on room air. Chest radiography demonstrated bilateral pulmonary edema consistent with acute decompensated heart failure. Troponin was negative, and BNP was markedly elevated at 1,847 pg/mL. Transthoracic echocardiography confirmed severely reduced left ventricular ejection fraction of 22% with global hypokinesis, consistent with his known ischemic cardiomyopathy. The patient was admitted to the cardiac care unit for intensive diuresis and hemodynamic optimization.

During the hospital course, the patient received aggressive intravenous diuretic therapy with furosemide, achieving negative fluid balance of approximately 8 liters over the first four days. Afterload reduction was optimized with uptitration of lisinopril and addition of carvedilol. Electrolytes were monitored closely given diuretic use, and potassium supplementation was provided to maintain appropriate levels. The patient's respiratory status improved significantly, with oxygen requirements weaning to room air by hospital day five. Repeat BNP on day six decreased to 612 pg/mL, reflecting improved hemodynamic status. His existing ICD remained stable throughout the admission with no arrhythmias detected on telemetry monitoring.

The patient was discharged on hospital day seven in stable condition with significant clinical improvement. He was counseled extensively on sodium and fluid restriction, medication adherence, and daily weights with instructions to report gains exceeding three pounds. Outpatient cardiology follow-up was arranged for one week post-discharge to reassess volume status and consider further guideline-directed medical therapy optimization. Home health nursing was arranged for medication reconciliation and monitoring. The patient demonstrated good understanding of his condition and expressed commitment to lifestyle modifications to prevent future decompensation.

Procedures Performed

IV diuresis with furosemide	—
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Medication Changes

Furosemide 40 mg BID	STARTED
Empagliflozin 10 mg daily	STARTED
Carvedilol 6.25 mg BID	ADJUSTED
Sacubitril/valsartan 49/51 mg BID	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	8 days
Urgency Tier	HIGH
Clinical Flags	30-day-readmission-risk, active volume overload at discharge, EF<40%

Dictated and electronically signed by:
Kelsey Harper, MD
NPI: 1939909168
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